



OFFICIAL

NMHS MHS POLICY & PROCEDURE

Physical Health Care of Mental Health Consumers

Scope (Audience)	All NMHS Mental Health Services (MHS) Staff
Scope (Area)	All NMHS MHS Inpatient and Community Areas
Exempt Services	Links Perth Children's Court, Youth Detention, Prison In-Reach, Dialectical Behavioural Therapy (DBT), CECAT, Neuroscience, Centre for Clinical Intervention (CCI)
Summary	Policy and procedure provides direction to North Metropolitan Health Services (NMHS) Mental Health Services (MHS) to ensure high quality provision of physical health care to mental health consumers by outlining the standards required to deliver a consistent, coordinated approach whilst receiving care from a NMHS MHS including Community Mental Health Services and Authorised Mental Health Inpatient Units
Risk Rating	Medium
Policy Standards	Clinical Governance (Std 1) Partnering with Consumers (Std 2) Medication Safety (Std 4) Comprehensive Care (Std 5) Communicating for Safety (Std 6) Recognising and Responding to Acute Deterioration (Std 8)
Policy Sponsor	Director of Medical Services MHSDHS
Policy Contact	Director of Medical Services MHSDHS
Version	V1.0
Review Date	27/03/2027

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

Physical health assessment, care and ongoing monitoring involves collaboration and partnership between clinical staff, primary health providers, consumers, and their family, carer, and/or personal support persons. This collaborative partnership plays a critical role in improving physical health outcomes for mental health consumers.

This document provides direction to North Metropolitan Health Services (NMHS) Mental Health Services (MHS) to ensure high quality provision of physical health care to mental health consumers by outlining the standards required to deliver a consistent, coordinated approach.

This policy aligns with:

- [Mental Health Act 2014](#) (WA)
 - [Charter of Mental Health Care Principles](#)
- [Chief Psychiatrist's Standards for Clinical Care](#) - Standard: Physical Health Care of Mental Health Consumers
- [MP 0155/21 – Statewide Standardised Clinical Documentation \(SSCD\) for Mental Health Services Policy](#)

2. Key Points

This policy is a mandatory requirement under s.241 of the [Mental Health Act 2014](#) (WA), [Schedule 1 – Charter of Mental Health Care Principles](#) and the [Chief Psychiatrist's Standards for Authorised Hospitals under the Mental Health Act 2014](#).

This policy applies to all mental health consumers receiving care from a NMHS Mental Health Service, including Community Mental Health Services and Authorised Mental Health Inpatient Units.

NMHS Mental Health Services primary aim and goal is to improve the mental health outcomes for individuals within the service. While the primary aim and specialty is the assessment and treatment of mental health conditions, it must be acknowledged that mental health consumers experience higher rates of physical ill health, either directly or indirectly related to their mental illness. It is imperative that mental health services consider physical health and wellbeing and potential implications of mental health treatments or mental illness on individuals physical health.

3. Risk

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Policy

In accordance with the [Mental Health Act 2014 \(WA\)](#) Schedule 1- Charter of Mental Health Care Principles (Principle 8: Co-occurring needs), mental health services must address the physical, medical and dental health needs of people experiencing mental illness and other co-occurring health issues, including physical and intellectual disability and alcohol and other drug problems.

Mental health consumers have a right to physical, medical, and dental health care that is equal to the healthcare provided to the general population.

Physical health care is to be delivered in a respectful, non-judgemental and trauma-informed manner that considers dignity, privacy, gender, sexuality and the consumer's preferences in accordance with the [MHA Charter of Mental Health Care Principles](#).

Consumers are to be provided with information regarding physical examinations in a language and manner they can understand. Accredited interpreter services are to be used to facilitate communication with consumers who are non-hearing or from culturally and linguistically diverse backgrounds in accordance with [MP 0051/17 WA DoH Language Services Policy](#).

The presence of a chaperone or support person (or both) is to be offered during the physical examination in accordance with the [NMHS Chaperone Policy](#).

Physical healthcare is to be planned and delivered in a manner that respects and where possible, incorporates cultural, spiritual, gender specific needs, religious beliefs, and practices.

Where the consumer is of Aboriginal descent, the consumer is to be offered the support of an Aboriginal Mental Health Worker to advocate for culturally appropriate care. With respect to physical health care of a patient who identifies as Aboriginal, staff must be particularly mindful of cultural needs with regards to trauma informed care, age, and gender specific considerations such as men's and women's business.

Physical health care needs are to be considered and included in the planning and provision of comprehensive care delivered or coordinated by the mental health service. Comorbid use of alcohol and other drugs and lifestyle choices are to be recognised in treatment planning.

All relevant information regarding a consumer's physical health care needs is to be communicated and documented accurately in a timely manner and in accordance with the [Statewide Standardised Clinical Documentation \(SSCD\)](#) and the [MHS Clinical Record Documentation Procedure](#).

In accordance with the [Guardianship and Administration Act 1990](#) - Guardians are to be included in clinical reviews and/or care planning meetings, to understand treatment goals, and support their decision making role.

NMHS MHS acknowledges and supports the principles outlined in the Western Australian [Carers Recognition Act 2004](#) and the associated Western Australian Carers Charter. In accordance with the [NMHS Carers Recognition Policy](#), MHS staff are to recognise the role of carers, family and support persons, and facilitates their involvement in the assessment and management of physical health care needs of consumers.

Where clinical care requirements extend beyond the scope of the mental health service, collaboration with alternative or specialist care delivery services is to be considered. Consumers

and their families are to be offered information regarding relevant supports and appropriately referred to other services for further care, as required.

5. Physical Health Screening and Monitoring Procedures

Staff must provide information to the consumer in a language and manner they can understand to obtain informed consent prior to undertaking physical examinations in accordance with the [Department of Health's MP 0175/22 Consent to Treatment Policy](#).

Physical health care should be responsive to individual consumer needs, preferences, gender identity, cultural background, language proficiency, and age to ensure equitable and accessible care.

Consumers prescribed psychotropic medications and identified as being at risk of metabolic syndrome, must be monitored. Monitoring incorporates the measurement height, weight, Body Mass Index (BMI), waist circumference and regular age-appropriate blood screening relative to medications prescribed.

For management and monitoring of Clozapine refer to:

- [MHS Medication: Clozapine Prescribing and Monitoring Policy and Procedure](#)
- [MHS Medication: Clozapine Coordinator Procedure – Clozapine Clinic Procedure](#)
- [MHS Medication: Clozapine Management by General Practitioners Procedure](#)

5.1 Inpatient Services – Adult, Older Adult & Forensics

Physical examinations and physical appearance forms are to be conducted/completed within **12 hours of admission**, ensuring the physical wellbeing of all consumers admitted are assessed and where appropriate referred to appropriate specialist clinicians.

Long stay patients require physical health examination and appearance forms to be completed annually or more frequently as required.

Documentation required:

- [Physical examination form \(webPSOLIS\)](#)
- [Physical appearance form](#) (SMHM904) (hard copy)
- [Oral health assessment](#) (for referral if required)
- Referral to specialist clinicians (if required)
- [Mental Health Assessment Form \(SMHMR902\)](#) includes a section - Physical Examination Summary (include physical examination key findings, information source, if referred to a GP)
- Any identified physical health care goals, needs, support or treatment plans should be incorporated into the [Treatment, Support and Discharge Plan](#) (TSDP)
- [Metabolic Monitoring form](#) (hard copy)

If a physical examination cannot be performed within 12 hours, or the consumer refuses a physical health assessment, this is to be clearly documented, and the treating consultant notified. Physical Health Assessment should be reoffered to the consumer at each subsequent clinical review and documented within the consumer's health record.

Discharge or Transfer of Care

Effective communication and good clinical handover of consumer information is critical to safe, well-integrated care. Any clinical information and investigations pertaining to psychotropic

medication management and ongoing monitoring recommendations must be provided to the receiving service and forwarded to the consumer's GP or primary care provider.

The treating team will ensure that the follow up service is provided with all relevant information to enable continued management of the consumer's physical health care needs. Documentation provided is to include:

- Care Transfer Summary
- Treatment Support and Discharge Plan and any other relevant care plans
- Medication Charts
- NaCS Discharge Summary

5.2 Community Mental Health Services – Adult, Older Adult, Child & Adult Forensics & Youth

Clinicians must seek and document (PMR03) the consumers consent to communicate with other health professionals and respect the consumers wishes should they decline/refuse consent.

The Physical Appearance Form is required to be completed within one (1) month of activation and annually thereafter.

Community services are to incorporate physical health needs into the consumer's three-monthly clinical review by asking the question (if appropriate) ***“do you have any physical health care or dental concerns”***:

- If consumer declines to respond due to severity of illness, document circumstances, reasoning and follow up accordingly.
- If the response is **YES**, facilitate effective communication and clinical handover with GP/primary or specialist care provider (see pathways below).
- If the response is **NO**, document and follow up at next three monthly review.

Community Mental Health Services will have access to a treatment room for the purposes of completing a physical examination and each community team will have access to the following standard equipment:

- Stethoscope
- Sphygmomanometer
- Thermometer
- Weighing Scales
- Tape Measure

A physical examination within Community Mental Health Services can be facilitated via one of three pathways.

Pathway 1:

Consumer has an existing General Practitioner (GP) or Primary Health Provider:

- Case Manager is to communicate with the GP to advise the consumer is receiving care from the service and seek any relevant information pertaining to the consumer's medical history.
- Request GP to conduct a physical examination and provide regular age appropriate metabolic monitoring and screening relative to antipsychotic medications prescribed and report back findings to the case manager/treating team.

- All relevant information and service events to be documented in PSOLIS/DMR/Medical Record. Any identified physical health care goals, needs, support, treatment, results/findings should be incorporated into the [SMHMR907 Treatment, Support, and Discharge Plan](#)(TSDP) form in WebPSOLIS.

Pathway 2:

Consumer does not have an existing a GP or Primary Health Provider:

- Case Manager must either assist the consumer to engage with a GP (such as a local non-governmental organisation (NGO), a [360 Street Doctor Community Health](#) or Aboriginal Medical Services ([AMS](#))) to complete a physical health check; **or**
- With the consumer's consent, arrange a suitably qualified staff member within the service to complete a physical health check documenting height, weight, Body Mass Index (BMI) and waist circumference.
- Provide the consumer with an age-appropriate metabolic monitoring blood test form relative to psychotropic medications prescribed. Provide support to consumer to facilitate blood test if required.
- Treating team in consultation with consumer are to follow up any abnormal results based on level of clinical risk. This may include:
 - sending the consumer to an emergency department;
 - assisting the consumer to make an appointment with a GP for further investigation; or
 - involving the consumer's carer/parent/guardian or support person if deemed necessary ensuring the agreed response is carried out as soon as reasonably necessary.
- All relevant information and service event to be documented in PSOLIS/DMR/Medical Record. Any identified physical health care goals, needs, support, treatment, results/findings should be incorporated into the [SMHMR907 Treatment, Support, and Discharge Plan](#)(TSDP) form in WebPSOLIS.

When conducting metabolic screening and monitoring within Community Mental Health Services, the treating team is to monitor and review dosage for youth, adolescent, older adults and Aboriginal consumers due to higher susceptibility to adverse effects from psychotropic medications. Written medication brochures can be accessed via the [Choice and Medication website](#) including education relating to:

- potential impact on the consumer's physical health
- interaction of lifestyle choices and substance use (e.g. smoking, caffeine, illicit substances, and non-prescribed medications) with psychotropics
- requirements for ongoing physical health monitoring

Pathway 3:

Consumer refuses Physical Health Examination:

- Should the consumer refuse physical health assessment, this is to be clearly documented, and the treating consultant notified.
- Physical Health assistance should be reoffered to the consumer at each clinical review.
- All relevant information and service event to be documented in PSOLIS/DMR/Medical Record. Any identified physical health care goals, needs, support, treatment, results/findings should be incorporated into the [SMHMR907 Treatment, Support, and Discharge Plan](#)(TSDP) form in WebPSOLIS.

6. Documentation

The Statewide Standardised Clinical Documentation ([SSCD](#)) is mandatory for all public mental health services and provides a suite of clinical documents to support the assessment and management of health. These documents are accessible via the Psychiatric Services Online Information System (PSOLIS) and in paper-based formats where required:

- When the physical examination is completed it must be documented using the [MH Physical Examination Form \(SMHMR903\) in WebPSOLIS](#).
- [Mental Health Assessment Form \(SMHMR902\)](#) includes a section - Physical Examination Summary (include physical examination key findings, information source, if referred to a GP)
- Any identified physical health care goals, needs, support or treatment should be incorporated into the [MH Treatment, Support and Discharge Plan \(TSDP\)](#).
- Refusals of consent, rationales, education, and alternative arrangements are to be clearly documented in the consumer's health record and on the relevant examination/screening/ [TSDP](#) form.
- If the consumer reports no physical health concerns, this should also be documented in the [TSDP](#).
- Evidence of communication with primary care provider (e.g. letters, emails, progress notes detailing phone conversations) must be documented in the medical record and clearly identifiable.

Department of Health SSCD Physical Health Care tools:-

- [Mental Health Metabolic Monitoring Form \(SMH001\)](#)
- [Mental Health Oral Health Checklist Form \(TMHMR992\)](#)

7. Roles and Responsibilities

Roles	Responsibilities
NMHS MHS Medical Co Directors	Have overall responsibility for clinical governance of NMHS Mental Health Services, ensuring staff comply with physical health care requirements of mental health consumers and implementation of this procedure and related policies.
NMHS MHS Operational Co Directors	• Are responsible for the operational management of the mental health service in consultation with Clinical Nurse Managers and/or Service Managers. • Are responsible for ensuring that treating teams have the appropriate equipment and resources to complete required screening and those governance systems are in place to ensure they are in working order or replaced as required.
NMHS MHS Consultant Psychiatrists	• Are responsible for the psychiatric, medical management of consumers in consultation with the multidisciplinary treatment team. • Are responsible as part of ongoing clinical review for ensuring that physical (including oral health) examination is completed as part of treatment planning and review and recording clinical information in the mental health clinical information data base (webPSOLIS).

NMHS MHS Case Managers	• Are responsible for the psychiatric, medical management of consumers in consultation with the multidisciplinary treatment team. • Are responsible as part of ongoing clinical review for ensuring that physical health care (including oral health) examination, is completed as part of treatment planning and review and recording clinical information in the mental health clinical information data base (webPSOLIS).
NMHS MHS Clinical Staff	Are responsible for the provision of mental health care within their scope of practice and in accordance with applicable legislation, policies, procedures and best practice standards.
All NMHS staff	Are responsible for: • Having knowledge of the policy documents relevant to their roles and how to access these documents. • Complying with mandatory requirements outlined in these documents. • Assisting with policy development/reviews wherever required. • Reporting to their immediate line manager any potential non-compliance or implementation issues.

8. Monitoring and Evaluation

Compliance with this policy is mandatory.

NMHS Mental Health Services (MHS) will ensure the effectiveness of this policy through ongoing monitoring, evaluation, and quality improvement initiatives.

A Physical Health Care Audit is conducted annually and includes compliance monitoring related to physical health, including:

- completion of SSCD Physical Examination within mandated time frame
- documentation of the SSCD Treatment Support and Discharge Plan
- In addition, monitoring and trending of SAC 1 or SAC 2 incidents relevant to physical health care, may prompt a review of this document, and escalation should occur to the policy sponsor.

This policy will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed every 3 years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

9. Related Documents

Legislation	• Mental Health Act 2014 (WA) • Criminal Law (Mental Impairment) Act 2023 (WA) • Guardian and Administration Act 1990 (WA)
WA Health mandatory policies	• MP 0155/21 Statewide Standardised Clinical Documentation for Mental Health Services Policy • MP 0062/17 Alcohol and Other Drug Withdrawal Management Policy • MP 0095/18 Clinical Handover Policy

	• MP 0134/20 Recognising and Responding to Acute Deterioration Policy • MP 0175/22 Consent to Treatment Policy • MP 0131/20 High Risk Medication Policy • MP 0104/19 Medication Review Policy
NMHS Policy documents	• NMHS Clinical Documentation Policy • NMHS Chaperone Policy
NMHS MHS Policy documents	• MH Medication: Management of High-Risk Medication. • MHS Medication: Clozapine Prescribing and Monitoring Policy and Procedure • MHS Medication: Clozapine Coordination Procedure – Clozapine Clinic • MHS Medication: Clozapine Management by General Practitioner Procedure • MH Recognising and Responding to Acute Deterioration (RRAD) Physiological and Mental State Procedure. • MHS Clinical Record Documentation Procedure • MHS Recognising and Responding to Acute Deterioration (RRAD) Physiological and Mental State Procedure (Inpatient) • MHS Inpatient Admission to Discharge Policy • MHS Community Admission to Discharge Policy • MHS Communication with General Practitioners (GP) and Shared Care Policy – Community Adult
OCP Standards	• Physical Health Care of Mental Health Consumers • The Chief Psychiatrist's Authorised Hospital Standards

10. Resources

This policy is informed by key documents that emphasise the importance of integrating physical and mental health care. These include:

- [National Safety and Quality Health Service \(NSQHS\) Standards \(second edition\)](#)
- [National Safety and Quality Health Service Standards – User Guide for Aboriginal and Torres Strait Islander Health](#)
- [The University of WA – Clinical guidelines for the physical care of mental health consumers.](#)
- [Fifth National Mental Health and Suicide Prevention Plan](#)
- [New South Wales Health - Physical Health Care for People Living with Mental Health Issues](#)
- [National Mental Health Commission Equally Well – Improving the physical health and wellbeing of people living with mental illness in Australia](#)
- [Royal Australian & New Zealand College of Psychiatrists \(RANZCP\) Clinical Guidelines and Publications.](#)
- [WA Aboriginal Health and Wellbeing Framework 2015 - 2030](#)
- [Department of Health – Information about psychotropic medications for consumers and their carers and families.](#)

11. Document Control

Version	Policy author	Summary of changes	TRIM Reference	Approval date
V1.0	Director of Medical Services	Initial publication of new policy and procedure	D/26/70813	25/03/2026

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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